



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : شيماء ياسين محمود سليمان
المحافظة : القليوبية
الحى / المدينة : شبرا الخيمة
العنوان : ممم
رقم البطاقة : 28606081400963 رقم التليفون : 01285079825
العمر (تاريخ الميلاد) : 08-06-1986 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

د.ا. / د.ا. / د.ا. / د.ا. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: شيماء ياسين محمود سليمان
National ID: 28606081400963



تقرير طبي

المسند

تحية طبية وبعد،،

بشأن تقرير طبي للمريض

إيماء للطبيب المقدم من

رقم طبي:

بقسم:

تنتهله بان توضح لهما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة التحويل :
--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound

الوقت :

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين





شيماء
ياسين محمود سليمان
مستلقة اسنكر بشوك جهنيم
شبرا القوية ثلث - القنوية

٢٨٦٠٦٠٨١٤٠٠٩٦٣

LC4430729

Doctor

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection.....

* C.I.D: severe lung sides, especially at Lt. Lt.

* For:

W6 Taxol 160mg Today & after ches

W7 Taxol 160mg on 26/6/2022

W8 Taxol 160mg on 3/7/2022

* Bone scan: requested & pending.

c.c.c 17/1/19 التاريخ: 40mg 1N x 30d.
الوقت: 500mg 2x2x30d.

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection.....

W9 taxol
Cph 10mg B
26/6/2022
W8 Taxol
Cph 3/7/2022

17/7/2022

W10
Cph 10mg B
26/6/2022

1 / 1 التاريخ:
الوقت:

Handwritten signature



امرا ل...
مرا ل...



chest:.....

Temp:.....

Oral mucosi

Canula site infection.....

* C/O: success lung aches, especially

✓ T/C:

W6 Taxol 160mg Today

W7 Taxol 160mg on 26/6/2020

W8 Taxol 160mg on 3/7/2020

✓ Bone scan: requested & pending

CC 17/19 التاريخ: 40mg 1x13

الوقت: Paracetamol 500mg 2x2

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection.....

13/7/2022

CC

W10

10x13

التاريخ:

الوقت:

Handwritten signature and date 15/7/2022.





مكتبة

Clinical Data

MEDICAL ONCOLOGY	SURGERY
11/1/12	11/1/12



المبادئ الثلاثة لصحة المرأة - معمل الباثولوجي

Patient Name	: 2 years 4 months / 2010-11-11
Patient No.	: 28606081400963
Path No.	: 243/22
Sex	: Female
Age	: 35 Y
Referring Doc:	

Pathology Report

Clinical Data:
Breast cancer.

Nature of the specimen:
WLE & sentinel lymph nodes biopsy

Gross Examination:

Four specimens were received:

1-WLE specimen measured 8x7x2.5cm. Cut section revealed a fairly defined greyish white mass measured 2.5x2.5x2cm.

-Distance from margins: superficial 0.1cm (least), deep 1cm, medial 2cm, lateral 0.3cm, superior 1cm, inferior 3cm.

2- Sentinel lymph nodes measured 3x1.5cm, including 4 lymph nodes, up to 1x0.9cm.

4-Extra lateral margin specimen measured 6x5cm

Microscopic examination:

- Histological type: Invasive duct carcinoma, with DCIS less than 5%.
- Grade 2 (differentiation + nuclear pleomorphism + mitosis): G (2+2+2)
- Positive perineural invasion
- Angiolymphatic: No
- Lymph node involvement: All sentinel lymph nodes were free (0/3).
- Resection Margins: All margins were free, including extra superficial and extra lateral margins
- Additional pathologic findings: No
- Frozen section: All sentinel lymph nodes were free (0/4). All margins were free, including extra superficial and extra lateral margins.

Verified by:

Patient Name : (الاسم / اسم المريض)

Patient No : 8676081407963

Age : 47

Sex : Female

Referring Doc : Dr. Y

Referring Dis :

Request No. : 132993

Request Date : 16/12/2021

Verification Date : 29/12/2021

Financial Category : اورام دالو السليم - ميناو

Department : Out Patient

IHC No :

Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Area Data

It breast mass

Nature of the specimen

For histology

For examination

M. Infiltrating gray white tissue cores up to 1cm, totally submitted. Sections prepared from paraffin block for immunohistochemistry staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), Ki-67 clone (4B5) & Ki67 clone (30-9)

For histology examination

For histological type (Hx)

For each Hx

For immunohistochemistry

For immunohistochemistry in situ Positive

For IHC

For breast mass, tru cut biopsy

For invasive duct carcinoma GII

For markers

For negative nuclear reaction in tumor cells (Allred score 0/8)

For negative nuclear reaction in tumor cells (Allred score 0/8)

For negative membrane reaction in tumor cells (score 0)

For 37 : 0 % nuclear reaction

Prof. Gamal Abdelaty

Verified by

Marwa fathy

Printing Date : 10-02-2022 10:23:22

Page 1 of 2

Printed By : marwa mohamed yousse



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي



Patient Name : الأسماء / أسماء ياسين محسن
Patient No. : 28606081400963
Path No. : 243/22
Sex : Female
Age : 35 Y
Referring Doc :

Request No. : 13000631
Request Date : 25/01/2022
Verification Date : 06/02/2022
Financial Category : عيولة - Out Patient
Department :
Registered by : marwa mohamed youss

Pathology Report

Diagnosis:

WLE & sentinel lymph nodes:

Invasive duct carcinoma grade II, with DCIS less than 5%.

All sentinel lymph nodes were free (0/4).

All margins were free.

Stage: pT2, pN0

Prof. Dr. Amal Hareedy
Dr. Abdulrahman Ibrahim

Verified by :

Printing Date : 10/02/2022 11:57:25

Page 2 of 2

Printing By : marwa mohamed youss



Total DLP (mGy.cm): 1097

Patient Name: Shairmaa Yasin Mahmoud Soliman	Date : 01/01/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 37 Year(s)
Medical ID: 48/2022	13758

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Compared to 06/02/2023

I Chest: -

- Right breast post operative status.
- Middle lobe anterior sub-pleural reticulaltions.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
مركز مراكز الطبية المتخصصة
مستشفى دار السلام
غرفة الأشعة

Total DLP (mGy.cm) : 1.097

Patient Name: Shaimaa Yasin Mahmoud Soliman	Date : 01/01/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 37 Year(s)
Medical ID: 48/2022	13758

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is noted.
- No abdominal or pelvic masses or collections.
- Dorsolumbar spondylosis.

Opinion:

- No CT evidence of metastatic pulmonary, hepatic or osseous disease.

Dr. Mohammed Mohsen, MSc

Dr. Amr Farouk, M.D.



Patient Name: Shaimaa Yasin Mahmoud Soliman	Date: 24/12/2023
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 48/2022	Pat. ID: 13758

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Report:

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- ACR C
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

Post right BCS status showing no evidence of recurrence (BIRADS 2)
Normal study of the left breast (BIRADS 1)

Signature Dr. Wafaa Ali



Patient Name: Shaimaa Yasin Mahmoud Soliman	Date : 06/02/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 36 Year(s)
Medical ID:	13758

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:-

Findings:

I Chest:-

- Right breast post operative status.
- Middle lobe anterior sub-pleural reticulations.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is noted.
- No abdominal or pelvic masses or collections.



Patient name: Shaimaa Yassin Mahmoud

Date: 12/12/2021

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Right breast UIQ middle third irregular area of focal asymmetry.
- Complementary ultrasound showed right breast UIQ irregular spiculated hypoechoic mass, with internal vascularity on color Doppler application, seen at 1 o'clock, 1.3cm from the overlying skin, 3.7cm from the nipple, measured 1.3x2.1cm, consistent to the palpable lump.
- No left breast suspicious cystic or solid mass lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific left axillary LN.
- only 2 in number axillary LNs, with cortical thickness 3mm, likely indeterminate.

Opinion:

- Right breast 1o'clock highly suspicious mass (BIRADS 4c), for histopathological correlation.
- No left breast suspicious findings (BIRADS 2)

Yours truly

Dr Engy Abdullah



Name : **Shaimaa Yaseen Mahmoud**

Date : **4/1/2022**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:-

1. Chest:

- Right breast upper central enhanced mass measuring about 23x21 mm with right axillary level 1 enlarged lymph nodes are seen the largest measures about 18x13 mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is seen.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Right adenexal hypodense cyst measures about 4x2.5cm
- No lytic or sclerotic bone lesions.

OPINION:

- ***Right breast enhanced mass with right axillary enlarged lymph nodes.***
- ***No CT evidence of pulmonary or hepatic metastatic deposits.***

Dr. Reham Anwar, MSc, FEBR

Dr. A. Gomaa, M.D.



Patient Name: Shaimaa Yasin Mahmoud	Date : 06/02/2023
Soliman	
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 48/2022	Pat. ID: 13758

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Report:

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- ACR C
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Signature Dr. Peter Nashat

06/02/2023 3:30 PM



شيماء
ياسين محمود سليمان
مستلقة اسنكر بشوك جهنيم
شبرا القوية ثلث - القتيوية

٢٨٦٠٦٠٨١٤٠٠٩٦٣

LC4430729

Doctor

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

* CXR: severe lung aches, especially at Lt. Lt.

* For:

W6 Taxol 160mg Today & after ches

W7 Taxol 160mg on 26/6/2022 (CXR)

W8 Taxol 160mg on 3/7/2022 (CXR)

* Bone scan: requested & pending.

c.c.c 17/1/19 التاريخ: 40mg 1N x 30d.

الوقت: 500mg 2x2x30d.

الأوامر

chest:.....
Temp:.....
Oral mucositis:.....
Canula site infection:.....

W9 Taxol

Cph/Omg B

26/6/2022

W8 Taxol

Cl Ph 3/7/2022

17/7/2022

W10

W11

W12

W13

W14

W15

W16

W17

W18

W19



chest:.....

Temp:.....

Oral mucosi

Canula site infection.....

* C/O: success lung aches, especially

✓ T/C:

W6 Taxol 160mg Today

W7 Taxol 160mg on 26/6/2020

W8 Taxol 160mg on 3/7/2020

✓ Bone scan: requested & pending

CC 17/19 التاريخ: 40mg 1x13

الوقت: Paracetamol 500mg 2x2

chest:.....

Temp:.....

Oral mucositis:

Canula site infection.....

13/7/2022

W10

10x13

التاريخ:

الوقت:

Handwritten signature and date 15/7/2022.



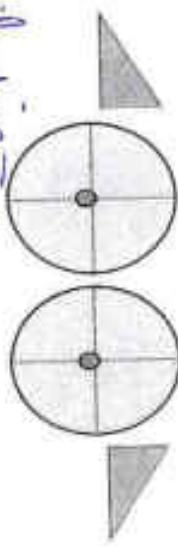


مملكة العربية السعودية
وزارة الصحة
لجنة صحة المرأة المصرية

لجنة صحة المرأة المصرية
مصر

C.C.C/1/12 MDT FORM

Name	سبحان, سحر
Age	25
MRN	C.C.C/1/12
ID	1510-09100
Co morbidity	HIV +ve (family history (parent))



C.A.C/1/12/0912 Clinical Data

Bx:	ITDC G2	SM:	1.2x1.2x1.2 cm at 12 o'clock
IHC:		Mets Workup:	-ve
ER:	-ve		
PR:	-ve		
Her2:	-ve		
KITV:	1.2x1.2 cm		
CT No Mo			

COMMENT:
For CESM $\rightarrow$ if unifocal
BCS & SNB, if multifocal or
multicentric $\rightarrow$ R-met

14-2-2012 BCS+SNB. done $\rightarrow$ CT No Mo
MDT OUTCOME & PLAN (2-5C)

for Adv. chemo ACX4 $\rightarrow$ Taxol X12
the PORT

Dr. S. S.

SIGNATURE		
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY
Dr. S. S.	Dr. S. S.	Dr. S. S.
PATHOLOGY		

Dr. S. S.

Patient Name : (الاسم / اسم المريض)

Patient No : 8676081407963

Age : 47

Sex : Female

Referring Doc : Dr. Y

Referring Dis :

Request No. : 132993

Request Date : 16/12/2021

Verification Date : 29/12/2021

Financial Category : اورام داء الثدي - ميثارة

Department : Out Patient

IHC No :

Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Lab Data

It breast mass

Nature of the specimen

For histology

For examination

M. Infiltrating gray white tissue cores up to 1cm, totally submitted. Sections prepared from paraffin block for immunohistochemistry staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), Ki-67 clone (4B5) & Ki67 clone (30-9)

For histology examination

Slide type (H&E)

Slide no.

For immunohistochemistry

For carcinoma in situ. Positive

For IHC

For breast mass, tru cut biopsy

For invasive duct carcinoma, GII

For markers

ER: Negative nuclear reaction in tumor cells (Allred score 0/8)

PR: Negative nuclear reaction in tumor cells (Allred score 0/8)

Ki-67: Negative membrane reaction in tumor cells (score 0)

Ki-67: 0 % nuclear reaction

Prof. Gamal Abdelaty

Verified by

Marwa fathy

Printing Date : 10-02-2022 10:23:22

Page 1 of 2

Printed By : marwa mohamed yousse



Patient Name : الأسماء / أسماء ياسين محمود
Patient No. : 28606081400963
Path No. : 243/22
Sex : Female
Age : 35 Y
Referring Doc :

Request No. : 13000631
Request Date : 25/01/2022
Verification Date : 06/02/2022
Financial Category : عيولة - Out Patient
Department :
Registered by : marwa mohamed youss

Pathology Report

Diagnosis:

WLE & sentinel lymph nodes:

Invasive duct carcinoma grade II, with DCIS less than 5%.

All sentinel lymph nodes were free (0/4).

All margins were free.

Stage: pT2, pN0

Prof. Dr. Amal Hareedy
Dr. Abdulrahman Ibrahim

Verified by :



Total DLP (mGy.cm): 1097

Patient Name: Shaimaa Yasin Mahmoud Soliman	Date : 01/01/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 37 Year(s)
Medical ID: 48/2022	13758

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Compared to 06/02/2023

I Chest: -

- Right breast post operative status.
- Middle lobe anterior sub-pleural reticulatlons.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.



Total DLP (mGy.cm) : 1.097

Patient Name: Shaimaa Yasin Mahmoud Soliman	Date : 01/01/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 37 Year(s)
Medical ID: 48/2022	13758

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is noted.
- No abdominal or pelvic masses or collections.
- Dorsolumbar spondylosis.

Opinion:

- No CT evidence of metastatic pulmonary, hepatic or osseous disease.

Dr. Mohammed Mohsen, MSc

Dr. Amr Farouk, M.D.



Patient Name: Shaimaa Yasin Mahmoud Soliman	Date: 24/12/2023
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 48/2022	Pat. ID: 13758

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Report:

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- ACR C
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

Post right BCS status showing no evidence of recurrence (BIRADS 2)
Normal study of the left breast (BIRADS 1)

Signature Dr. Wafaa Ali



Patient Name: Shaimaa Yasin Mahmoud Soliman	Date : 06/02/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 36 Year(s)
Medical ID:	13758

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:-

Findings:

I Chest:-

- Right breast post operative status.
- Middle lobe anterior sub-pleural reticulations.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is noted.
- No abdominal or pelvic masses or collections.



Patient name: Shaimaa Yassin Mahmoud

Date: 12/12/2021

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Right breast UIQ middle third irregular area of focal asymmetry.
- Complementary ultrasound showed right breast UIQ irregular spiculated hypoechoic mass, with internal vascularity on color Doppler application, seen at 1 o'clock, 1.4cm from the overlying skin, 3.7cm from the nipple, measured 1.3x2.1cm, consistent to the palpable lump.
- No left breast suspicious cystic or solid mass lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific left axillary LN.
- only 2 in number axillary LNs, with cortical thickness 3mm, likely indeterminate.

Opinion:

- Right breast 1o'clock highly suspicious mass (BIRADS 4c), for histopathological correlation.
- No left breast suspicious findings (BIRADS 2)

Yours truly

Dr Engy Abdullah



Name : **Shahmua Yaseen Mahmoud**

Date : **4/1/2022**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:-

1. Chest:

- Right breast upper central enhanced mass measuring about 23x21 mm with right axillary level 1 enlarged lymph nodes are seen the largest measures about 18x13 mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is seen.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Right adenexal hypodense cyst measures about 4x2.5cm
- No lytic or sclerotic bone lesions.

OPINION:

- ***Right breast enhanced mass with right axillary enlarged lymph nodes.***
- ***No CT evidence of pulmonary or hepatic metastatic deposits.***

Dr. Reham Anwar, MSc, FEBR

Dr. A. Gomaa, M.D.



Patient Name: Shaimaa Yasin Mahmoud	Date : 06/02/2023
Soliman	
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 48/2022	Pat. ID: 13758

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Report:

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

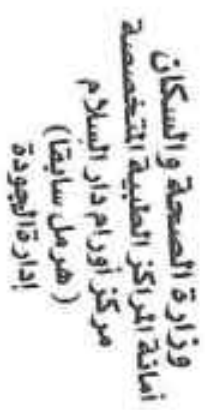
- ACR C
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Signature Dr. Peter Nashat

06/02/2023 3:30 PM



اسم المريض :
الرقم الطبي :

2019/2/1

Temp:

Canula site infection.....

توزيع الحطيب :

التاريخ:
الوقت:

125/51

Temp:.....

Canula site infection.....

توزيع الطليبي :-

التاريخ:
الوقت:



التاريخ المرضي PERSONAL HISTORY

Patient name: السيد ياسين محمد Mr.no:

Age: 35 Sex: ☐ Male ☐ Female

Occupation:

Marital History : ☐ M ☐ S ☐ W ☐ D

Children: ☐ Yes ☐ No number 4 offspring

Special Habits: ☐ Smoking ☐ Coffee ☐ Alcohol

Allergy : ☐ Yes ☐ No Comment:

Adverse drug reaction: ☐ Yes ☐ No Comment:

Complaints:

Present History: RT CTZ No Hb WBC Cell

Triple Negative

BCS was done + sentinel LN

PIZ No/14 Hb

MDT for adJ 4AC -> 12 wks fared.

MDT PART

Past History (hospital admission & surgery):

HIV

+ve family

- surgical:
- Diabetes mellitus:
- Blood Transfusion:

Family History:

+ve family history

grand mother (breast)

Psychosocial History:

Checked & Reviewed by Dr. MD 'A' 5

Date: Time: C-CC/117



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)

الفحص الطبي PHYSICAL EXAMINATION

B.P:..... Pulse:..... R.R:..... Temp:..... Weight:..... Height:.....

Pain: ☐ Yes ☐ No Specify:

Surface Area:

Head & Neck:
Upper Limbs:

Chest:

Breast:

Abdomen & pelvis:

Lower limbs:

Other:

Swelling:

Site:

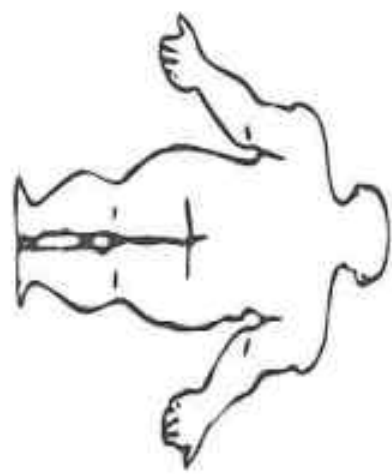
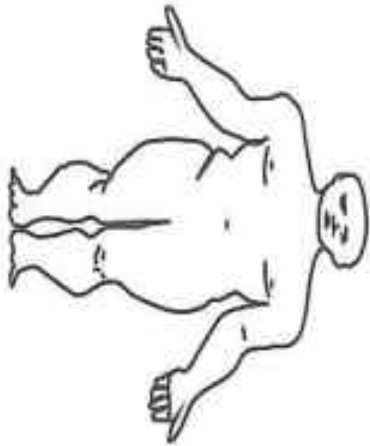
Size:

Shape:

Consistency:

Mobility:

Skin overlying:



Provisional Diagnosis:

Associated Risk Factors:

Checked & Reviewed by Dr.

Date: Time:



أوامر الطبيب Doctor's Orders

2019

chest:.....373 | 2024

Temp: 72.1

Canula site infection.....

✓
Chen W. ~~for~~ me

phlAfg fugez

C3 AC

توزيع الطيب
31/3/2017

॥३॥

الوقت:

12/9/21

$$\underline{\underline{WSC = 1.22}}$$

931516777

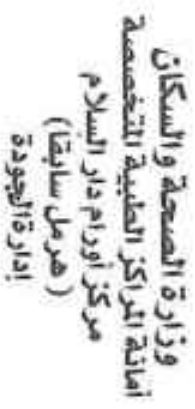
Temp:.....
Oral mucositis:.....

Canula site infection.....

بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ

المراجع:

三



اسم المريض :
الرقم الطبي :

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection.....

التاريخ:
 الموقع:

السلامة
التي

وہاں

```
chest:.....
```

Temp:

Oral mucositis:.....

Canula site infection.....

توزيع الطليعة *

التاريخ:
الوقت:



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :

الرقم الطبي :

الأوامر

Chest:.....

Temp: 28.9/36.2

Oral mucositis: Check

Canula site infection

MRSA

Pen & TMC

Triple-IT

KIT

التاريخ:

الوقت:

توقيع الطبيب :

الأوامر

Chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection:.....

CTCAP

Pen & TMC

Triple-IT

التاريخ:

الوقت:

توقيع الطبيب :



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :

الأوامر

Chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection.....

Repharm

مراجعة الأدوية

التاريخ: ٢٠٢٤ / ١١ / ٢٤
الوقت: ١٢:٠٠

توقيع الطبيب: [Signature]

الأوامر

Chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection.....

CS III PN

CS III allanhera op

stop chemo & refer for BOP

١ = XSX1

٢ = ١

٢٠٢٤ / ١١ / ٢٤

١٢:٠٠

توقيع الطبيب: [Signature]

التاريخ: ٢٠٢٤ / ١١ / ٢٤
الوقت: ١٢:٠٠